

Pennsylvania Health Insurance Marketplace: 2026 Verified Summary

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Enrollment Decline

Pennie enrollment peaked at approximately 497,000 in 2025 (PHIEA, 2026a). Open Enrollment 2026 closed on January 31, 2026, with 486,000 enrollees — a loss of 85,000 people during the enrollment period itself, or roughly 1 in 5 enrollees (PHIEA, 2026b). Attrition continued afterward. As of July 1, 2026, total enrollment stood at 431,270; since the close of Open Enrollment, an additional 94,000 consumers had dropped coverage, bringing cumulative cancellations for the year to over 177,000 (PHIEA, 2026a).

Date	Enrollment	Note
2025 (peak)	~497,000	Record high under enhanced tax credits
Feb 1, 2026	486,000	End of Open Enrollment 2026 (85,000 terminated during OE)
Jun 1, 2026	443,024	160,000+ cumulative cancellations
Jul 1, 2026	431,270	177,000+ cumulative cancellations

Why: Expiration of Enhanced Premium Tax Credits

Enhanced Premium Tax Credits (EPTCs), worth roughly \$600 million annually to Pennsylvania enrollees, expired December 31, 2025, after Congress did not extend them (PHIEA, 2026a). Their expiration restored the 400% Federal Poverty Level eligibility cliff — individuals earning above roughly \$62,600 (about \$84,600 for a couple) no longer qualify for any federal premium assistance (PHIEA, 2026c). Pennie reports enrollees who kept coverage saw premiums increase by an average of 102%, and separately, ACA Signups reported in January 2026 that average net premiums

had more than doubled for the year (PHIEA, 2026a; ACA Signups, 2026).

Premium Rate Filings (Individual Market)

The Pennsylvania Insurance Department (PID) approved a statewide weighted-average gross premium increase of 21.5% for 2026, while denying \$50.1 million in unjustified requested increases during its review. PID attributed the increases to the EPTC expiration together with rising prescription drug costs and utilization (PID, 2026).

Carrier	Requested	Approved
Ambetter Health of Pennsylvania	30.1%	37.8%
UPMC Health Plan	16.3%	24.8%
UPMC Health Options	11.7%	20.2%
Keystone Health Plan East	23.5%	22.0%
Capital Advantage Assurance	26.0%	24.6%

Carrier	Requested	Approved
Oscar Health Plan of PA	21.6%	23.1%
Highmark Benefits Group	—	21.0%
Jefferson Health Plans PPO (Partners Insurance)	—	20.5%
QCC Insurance Company	16.7%	15.2%
Geisinger Quality Options	16.2%	13.8%
Geisinger Health Plan	14.1%	11.6%
Partners Insurance Company	-10.1%	-10.1% (sole decrease)

(PID, 2026)

County-Level Attrition

Pennie's official county termination data for Open Enrollment 2026 (through February 1, 2026) shows Schuylkill County with the highest termination rate in

the state at 24%. Of the top 20 highest-attrition counties, 15 (75%) are classified as rural (PHIEA, 2026d).

County	Termination %	Designation
Schuylkill	24%	Rural
Lehigh	22%	Urban
Northampton	22%	Urban
Lancaster	22%	Urban
Greene	22%	Rural
Fulton	22%	Rural
Tioga	21%	Rural
Monroe	21%	Rural
Franklin	21%	Rural
Juniata	20%	Rural
Mifflin	20%	Rural
Beaver	20%	Urban
Snyder	20%	Rural
Adams	19%	Rural

County	Termination %	Designation
Lebanon	19%	Rural
Carbon	19%	Rural
Blair	19%	Rural
York	19%	Rural
Cumberland	19%	Urban
Clearfield	19%	Rural

(PHIEA, 2026d)

Schuylkill County's high attrition coincided with the withdrawal of two of its three ACA carriers — Ambetter Health and Partners Insurance Company both removed the county from their 2026 coverage maps, leaving UPMC as the sole remaining carrier (PID, 2026). County residents faced an average monthly premium increase of 186%, or about \$347 more per person per month (Pottsville Republican Herald, 2026).

Consumer-Reported Affordability

A survey of 424 Pennsylvanians conducted by the Pennsylvania Health Access Network (PHAN) and the Pennsylvania Association of Community Health

Centers, published April 29, 2026, found a sharp decline in perceived affordability year-over-year (PHAN, 2026).

Perception	2025	2026
Unaffordable	38%	62%
Affordable	38%	26%
Very affordable	24%	12%

Additional survey findings: half of respondents said their plan doubled or tripled in cost for 2026; more than 1 in 6 said they planned to drop coverage; nearly 1 in 4 said they would downgrade to a cheaper plan with less coverage (PHAN, 2026).

The State Health Insurance Affordability Program

Act 54 of 2024 (the fiscal code for Budget Year 2024–25) established the State Health Insurance Exchange Affordability Program, which would provide state-funded premium assistance stacked on top of federal subsidies, similar to programs in New Jersey and California (Pennie/PHIEA, 2026e; HealthInsurance.org, 2026). The program has never been funded. As of the most recent confirmed

reporting (May 2026), no legislative appropriation had been made, and Pennie and PID were continuing to advocate for funding (ForHealthInsurance.com, 2026; PHIEA, 2026e).

No Surprises Act — Consumer Protections

The Pennsylvania Insurance Department administers state-level enforcement of the federal No Surprises Act, which protects patients from unexpected out-of-network medical bills in situations largely outside their control (PID, 2026f). Key protections, per PID's consumer materials:

- **Emergency services** — must be covered without prior insurance approval, regardless of whether the hospital, outpatient department, ambulatory surgical center, or air ambulance provider is in-network (ground ambulance is not covered by the Act) (PID, 2026f).
- **Non-emergency services at an in-network facility** — if a patient chooses an in-network facility but an individual provider there (e.g., a lab or pathologist) turns out to be out-of-network, services must still be covered at in-network rates (PID, 2026f).
- Consumers who believe they've received a surprise bill can submit a **No Surprises Bill**

Review Request through PID's consumer portal, or contact the Consumer Services Bureau at 1-877-881-6388 (TTY/TDD: 717-783-3898) (PID, 2026g).

Limitations

- Figures reflect the most recent confirmed data as of August 3, 2026. Enrollment and cost figures continue to update on Pennie's affordability dashboard; check pennie.com/affordability directly for anything more current.
- No peer-reviewed meta-analysis of Pennsylvania health insurance coverage for 2026 was located. This document is a cross-sectional compilation of primary-source administrative and survey data, not a meta-analysis.
- Figures such as an "ROI" for the unfunded State Affordability Program, specific peer-state dollar comparisons, and projected enrollee-restoration counts appeared in earlier, unverified drafts of this conversation and are intentionally omitted here because no primary source for them was found.

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